CASE STUDY: ECLAMPSIA

Clinical case summary and step-by-step management notes

1. Patient Assessment

Assessment	Recorded finding
Blood pressure (BP)	170/106 mmHg
Urine protein	2+
Respiratory rate (RR)	24 breaths/min
Pulse	80 beats/min
Reflexes	1+
Urine collected (catheter)	140 mL
Fetal heart rate (FHR)	144 beats/min (handwriting appears to read 144)
Gestational age	38 weeks
Labour	No signs of labour

Provisional diagnosis written in the notes: Eclampsia with severe hypertension.

2. Management: Step by Step

STEP 1: Call for Help

- Call the obstetric and emergency teams.
- Do not leave the patient alone.
- Prepare the emergency equipment.

STEP 2: Airway and Breathing

- Place the patient in the left lateral position.
- Maintain a clear airway.
- Give oxygen if needed.

STEP 3: Protect and Monitor the Patient

• Protect the patient from injury during a seizure.

• Check and monitor BP, pulse rate (PR), respiratory rate (RR), and oxygen saturation.

STEP 4: Control Convulsions

• Administer magnesium sulfate (MgSO■) according to the approved clinical protocol.

ECLAMPSIA MANAGEMENT

Continuation: maternal and fetal care, delivery planning, and medication notes

STEP 5: Control Blood Pressure

• Give labetalol or hydralazine as prescribed.

STEP 6: Monitor Mother and Baby

• Monitor the mother and the baby.

• Monitor urine output and fetal heart rate (FHR).

STEP 7: Plan Delivery

• Stabilize the mother first, then plan delivery.

STEP 8: Continue Post-Delivery Monitoring

• Continue monitoring the mother and baby.

• Continue MgSO■ for 24 hours after delivery or after the last seizure, whichever occurs later, as directed by the clinical protocol.

3. Medication Notes: Magnesium Sulfate (MgSO■)

Loading dose (as written in the handwritten notes): 4 [unit unclear] IV + 10 [unit unclear] IM; 5 [unit unclear] in each buttock.

Maintenance dose (as written): 5 [unit unclear] IM every 4 hours, according to the prescribed regimen.

IMPORTANT SAFETY NOTE: The dose units in the original handwriting are unclear and may have been misread. They have deliberately been marked as unclear rather than guessed. This document is a typed study-note transcription, not a treatment order. Confirm every medicine, dose, route, and timing with your instructor and the current hospital protocol before clinical use.

Abbreviations: BP = blood pressure; RR = respiratory rate; PR = pulse rate; FHR = fetal heart rate; IV = intravenous; IM = intramuscular; MgSO■ = magnesium sulfate.